

PARAS[®] TABLET

Ingredient(s):

Each tablet contains:

Chlorzoxazone	250mg
Paracetamol	300mg

Pharmacology (Summary of Pharmacodynamic and Pharmacokinetic):

Pharmacodynamic:

1. Paras Tablet combine the muscle relaxant effect of Chlorzoxazone with Paracetamol, a peripherally acting analgesic.
2. Data available from animal experiments as well as in human studies indicate that Chlorzoxazone acts primarily at the level of the spinal cord and subcortical areas of the brain where it inhibits multisynaptic reflex arcs involved in producing and maintaining skeletal muscle spasm of various etiology.
3. Paracetamol (acetaminophen) provides analgesic action to supplement that which results secondarily from muscle relaxation.

Pharmacokinetic:

1. Following oral administration, both Chlorzoxazone and Paracetamol are rapidly absorbed. Mean drug plasma concentrations reach a peak level in the majority of subjects in 45 to 90 minutes.
2. The plasma elimination half-life is about 1 hour for Chlorzoxazone and ranges from 1.5 to 3.5 hours for Paracetamol.
3. Metabolism is rapid, the principal metabolites are conjugates of glucuronic acid which are excreted primarily in the urine. Less than 1% of an administered dose of Chlorzoxazone and less than 4% of an administered dose of Paracetamol are excreted unchanged in the urine in 24 hours. Only traces of unchanged drug are excreted through the bile into the feces.

Indication(s):

Indicated as adjunct to other measures, such as rest and physical therapy, for relief of pain and muscle spasm associated with acute, painful musculoskeletal conditions.

Dosage and Administration

The usual adult dose is 1 -2 tablets four times a day.

To be dispensed on physician's prescription.

Contraindication:

Hypersensitivity to its components.

Precaution(s) / Warning(s):

1. Paras Tablet should be used with caution in patients with liver dysfunction or a history of allergic reactions to its components.
2. Patients using this drug should be cautioned about driving a car or operating potentially hazardous machinery. They may become drowsy or show impaired mental or physical abilities while taking this medication.
3. Safe use of this product during pregnancy and lactation has not been established. It should be given to women who suspect pregnancy or to pregnant women and nursing mothers only when clearly needed.
4. Allergy alert:
Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash.
These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Drug interaction(s):

Patients receiving antipsychotics, anti-anxiety agents or other CNS depressants (including alcohol) concomitantly with this drug may exhibit an additive CNS depression. When such combined therapy is contemplated, the dose of one or both agents should be reduced.

Side Effect(s) / Adverse Reaction(s):

Gastrointestinal disturbances may occasionally occur; in rare instances this drug has been associated with gastrointestinal bleeding. Drowsiness, dizziness, lightheadedness, malaise, or overstimulation may occasionally occur. Respiratory depression may occur in high dosage of Chlorzoxazone. Rarely, allergic-type rashes, petechiae, or ecchymoses may occur. Angioneurotic edema or anaphylatic reactions are extremely rare. There is no evidence that this drug will cause renal damage. The administration of this drug or other Chlorzoxazone products was suspected of causing liver damage.

Cutaneous hypersensitivity reactions including skin rashes, angiodema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.

Symptoms and Treatment for Overdosage, and Antidote(s):

Chlorzoxazone :

Initially, gastrointestinal disturbances such as nausea, vomiting, or diarrhea together with drowsiness, dizziness, lightheadedness or headache may occur. Early in the cause, there may be malaise or sluggishness followed by marked loss of muscle tone, making voluntary movement impossible. Respiratory depression may occur with rapid, irregular respiration and intercostal and substernal retraction.

Paracetamol:

In acute overdosage, dose-dependent, potentially fatal hepatic necrosis is the most serious effect. Renal tubular necrosis, hypoglycemia, coma, and thrombocytopenia may also occur. Early symptoms following a potentially hepatotoxic overdose may include gastrointestinal irritability, nausea, vomiting, anorexia, diaphoresis and general malaise. Clinical and laboratory evidence of hepatic toxicity may not become evident until 48 to 72 hours post-ingestion.

If overdosage occurs, the patient should undergo gastric lavage. Specific therapy with an antidote such as Acetylcysteine or Methionine and activated charcoal to absorb any remaining drug after emptying the stomach, has been suggested.

Storage Condition(s):

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

Shelf-Life:

3 years from the date of manufacture.

Product Description and Packing(s):

A light orange round tablet with the diameter of 13.5 mm, one side impressed with a circle and a dot in the middle “  ”

Plastic bottle of 1000's and 1500's (for export only)

Blister packing of 10's x 10 and 10's x 100

This preparation contains PARACETAMOL. Do not take any other Paracetamol containing medicines at the same time.



Manufacturer and Product Registration Holder:
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